

Policy on Organ Transplant Programme

«Hospital Name»

Document control

Document No.	HCO/COP/POL/15	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Transplant Coordinator Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

The organ transplant programme is carried out safely, legally, and ethically, and every hospital — whether or not it performs transplants — raises awareness of organ donation.

1. Purpose

This policy explains how «Hospital Name» runs its organ transplant programme in line with the Transplantation of Human Organs Act, guides transplant care with clinical practice guidelines, counsels recipients and donors before transplantation, and raises awareness of organ donation.

This policy does not cover general informed consent procedures in detail — those are covered in other hospital policies.

2. Scope

Sections 5.1 through 5.3 of this policy apply only where «Hospital Name» provides organ transplantation services. Section 5.4 (organ donation awareness) applies to «Hospital Name» regardless of whether it provides transplantation.

3. Policy standards

Where «Hospital Name» provides organ transplantation, it runs the programme in consonance with the Transplantation of Human Organs Act and ethical practice, guides transplant care through documented clinical practice guidelines and individualised care plans, and counsels recipients and donors before transplantation using trained counsellors.

«Hospital Name» takes measures to raise organ donation awareness regardless of whether it transplants organs.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not run an organ transplant programme without valid regulatory licences, without medical professionals permitted by the appropriate authorities, without a designated transplant coordinator, and without submitting required reports to the appropriate authorities within the defined timeframe.
- Do not provide transplant care without documented clinical practice guidelines covering indication for transplant, recipient fitness, donor fitness, education and consent, organ recovery, and organ compatibility, and without organ-specific guidelines for the pre-transplant, transplant, and discharge phases.
- Do not proceed with organ transplantation without qualified or trained counsellors having counselled both the recipient and the donor beforehand, covering benefits, risks, immunosuppression, and monitoring and follow-up.
- Do not skip taking measures to raise organ donation awareness, or skip counselling the family of a brain-dead patient about organ transplant where appropriate, done in a sensitive and courteous manner — this applies to «Hospital Name» whether or not it transplants organs.

5. What we do

5.1 Run the transplant programme legally and ethically

Where «Hospital Name» provides organ transplantation, it holds the required regulatory licences and follows the Transplantation of Human Organs Act. Only medical professionals permitted by the appropriate authorities take part in the programme. A designated transplant coordinator is in place. Required reports are submitted to the appropriate authorities within the timeframe the Act defines.

5.2 Guide transplant care with clinical practice guidelines

Written guidance based on standard treatment guidelines or sound clinical practice covers six areas: indication for transplant, recipient fitness, donor fitness, the education and consent process, organ recovery, and organ compatibility. Organ-specific clinical practice guidelines are documented for three phases — pre-transplant, transplant, and discharge. Individualised care plans guide each transplant patient's care; it's preferable for departments to also develop care pathways. A multi-disciplinary team is in place, including nurses, a dietician, rehabilitation services, a psychologist or medical social worker, and an infection prevention and control specialist. Resources specifically needed for transplant — consumables, pharmaceutical supplies, and diagnostic requirements — are made available.

5.3 Counsel recipients and donors before transplantation

Qualified or trained counsellors are available to counsel the recipient and donor before transplantation. Counselling covers the benefits and risks of donation and transplantation, and for recipients, immunosuppression and the monitoring and follow-up it requires. Counselling could take place during an interview with the Local Authorisation Committee. Evidence of counselling is recorded and kept, both in the required statutory formats and in «Hospital Name»'s own records.

5.4 Raise organ donation awareness

«Hospital Name» takes measures to raise public awareness of organ donation — for example, standees or handouts — and addresses common myths about it. This applies whether or not «Hospital Name» performs transplantation. Whenever a patient is declared brain dead, the family is counselled about organ transplant where appropriate, in a very sensitive and courteous manner.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced.
Transplant Coordinator	Designated coordinator for the organ transplant programme; ensures regulatory licences are current, that programme reports are submitted within the defined timeframe, and that clinical practice guidelines and individualised care plans are in place.
Transplant surgeons / treating doctors	Ensure transplant care is guided by CPGs and individualised care plans; participate in counselling of recipients and donors; confirm counselling is completed before transplantation.
Counsellors	Counsel recipients and donors before transplantation, covering benefits, risks, immunosuppression, and monitoring/follow-up; record evidence of counselling in statutory and hospital formats.
All clinical staff	Participate in organ donation awareness measures; counsel families of brain-dead patients in a very sensitive and courteous manner where appropriate.

7. Quality monitoring

Monitoring area	What is monitored
-----------------	-------------------

Regulatory compliance	Regulatory licences current; medical professionals hold required authority; transplant coordinator designated; programme reports submitted within the timeframe defined by the Act
Clinical practice guidelines	Written guidance covering all six content areas in place; organ-specific CPGs for pre-transplant, transplant, and discharge phases documented
Individualised care plans	Care plan documented for each transplant patient
Counselling	Evidence of counselling by qualified or trained counsellors recorded in statutory formats and hospital records before transplantation for each case
Organ donation awareness	Awareness measures in place (e.g., standees, handouts, myth-addressing content); brain-dead family counselling documented where appropriate

8. Training and staff acknowledgement

All staff involved in the organ transplant programme and in organ donation awareness activities shall be familiar with the requirements of this policy, including the conditional scope split (Sections 5.1 through 5.3 for transplant hospitals only; Section 5.4 for all), the six CPG content areas, the three CPG phases, counselling-before-transplantation timing, and the very sensitive and courteous manner required for brain-dead family counselling.

I have read the Policy on Organ Transplant Programme of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to all transplant programme staff, treating doctors, nurses, counsellors, and quality staff at «Hospital Name». Section 5.4 is distributed to all clinical staff.

10. Abbreviations

Abbreviation	Meaning
COP	Care of Patients (NABH Hospitals chapter)
CPG	Clinical Practice Guideline

IPC	Infection Prevention and Control
LAC	Local Authorisation Committee
NABH	National Accreditation Board for Hospitals and Healthcare Providers

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.15.a	CORE	Section 5.1 addresses regulatory licences, adherence to the Transplantation of Human Organs Act, professional authorisation by appropriate authorities, designated transplant coordinator, and report submission within the timeframe the Act defines. All five requirements are mandatory. CORE — non-compliance not acceptable for accreditation. Applies only where transplantation is provided.
COP.15.b	Commitment*	Section 5.2 addresses written guidance covering all six mandatory content areas (indication for transplant, recipient fitness, donor fitness, education and consent process, organ recovery, organ compatibility), organ-specific CPGs for all three phases (pre-transplant, transplant, discharge), individualised care plans, MDT composition, and resource availability. Applies only where transplantation is provided.
COP.15.c	Commitment	Section 5.3 addresses counselling of recipients and donors by qualified or trained counsellors before

		transplantation, covering benefits, risks, immunosuppression, and monitoring/follow-up. Evidence recorded in statutory formats and hospital records. Applies only where transplantation is provided.
COP.15.d	CORE	Section 5.4 addresses organ donation awareness measures and brain-dead patient family counselling where appropriate, in a very sensitive and courteous manner. Applies universally to all organisations regardless of whether transplantation is provided. CORE — non-compliance not acceptable.

12. Required Records/Evidence Checklist

Regulatory compliance (COP.15.a)

- Copies of current regulatory licences for the organ transplant programme.
- Documentary evidence that medical professionals hold the required authority from the appropriate authorities to participate in the programme.
- Designation record or letter for the transplant coordinator.
- Programme reports submitted to the appropriate authorities within the timeframe the Act defines — submission records or acknowledgements.

Clinical practice guidelines and care plans (COP.15.b)

- Written guidance covering all six content areas: indication for transplant, recipient fitness, donor fitness, education and consent process, organ recovery, and organ compatibility.
- Organ-specific CPGs for the pre-transplant, transplant, and discharge phases.
- Individualised care plans documented for each transplant patient.

Counselling (COP.15.c)

- Evidence of counselling in statutory formats and hospital records for each case — confirming counselling took place before transplantation.

Organ donation awareness (COP.15.d)

- Evidence of awareness measures in place (e.g., standees, handouts, myth-addressing content).
- Documentation of family counselling following a brain-dead patient declaration, where appropriate.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.15.
2. Transplantation of Human Organs Act (as amended) — governing legislation for organ transplant programmes in India.

Disclaimer

This policy reorganises the supplied COP.15 objective elements and approved plain-language content into policy format. No formal stop-work section is included — COP.15 is not in the accreditation stop-work proposals. Operating an organ transplant programme without valid regulatory licences or without medical professionals permitted by the appropriate authorities constitutes a statutory violation under the Transplantation of Human Organs Act — a legal compliance pre-condition, not merely an accreditation requirement. The conditional scope split is preserved exactly: Sections 5.1 through 5.3 apply only where transplantation is provided; Section 5.4 applies universally to all organisations. The six CPG content areas, the three CPG phases, the counselling-before-transplantation timing, and the very-sensitive-and-courteous manner for brain-dead family counselling are all preserved exactly. The Transplantation of Human Organs Act is named as the statutory reference in Section 5.1, the Non-negotiable rules, and Section 13 References.